

City General Hospital

Department of Nephrology

Patient Name:	Mohammed Farouk	MRN:	MGH-2024-11042
Age / Sex:	68 years / Male	Date:	05-Mar-2026
DOB:	03-Jan-1958	Physician:	Dr. Sunita Rao, DM (Nephrology)

NEPHROLOGY OUTPATIENT CONSULTATION

Chief Complaint: Leg swelling, fatigue, decreased urine output, and foamy urine for 4 months.

History of Present Illness: Mr. Mohammed Farouk is a 68-year-old male with a 15-year history of Type 2 Diabetes and 10 years of hypertension, presenting with progressive bilateral leg swelling, marked fatigue, reduced urine output, and frothy urine. He notes increasing breathlessness on exertion over 2 months. No dysuria or haematuria. Currently on Metformin 500 mg BD, Glipizide 5 mg daily, Amlodipine 10 mg, and Losartan 50 mg.

RENAL FUNCTION TESTS

Test	Result	Normal Range	Unit	Flag
Serum Creatinine	2.8	0.7 – 1.2	mg/dL	HIGH ↑↑
Blood Urea Nitrogen (BUN)	42	7 – 20	mg/dL	HIGH ↑
eGFR (CKD-EPI)	31	> 60	mL/min	LOW ↓↓
Uric Acid	8.4	3.5 – 7.2	mg/dL	HIGH ↑
Serum Potassium	5.6	3.5 – 5.0	mEq/L	HIGH ↑
Serum Sodium	133	135 – 145	mEq/L	LOW ↓
Serum Phosphate	5.2	2.5 – 4.5	mg/dL	HIGH ↑
Serum Calcium	7.8	8.5 – 10.5	mg/dL	LOW ↓
Haemoglobin	9.4	13.5 – 17.5	g/dL	LOW ↓
Urine Protein/Creatinine	2.8	< 0.2	ratio	HIGH ↑↑
Urine Microalbumin	480	< 30	mg/g	HIGH ↑↑
HbA1c	9.1	< 5.7	%	HIGH ↑
Intact PTH	118	15 – 65	pg/mL	HIGH ↑

CLINICAL NOTES

eGFR of 31 mL/min classifies this patient as CKD Stage 3b (G3b). The combination of long-standing diabetes and hypertension with severe proteinuria (UPCR 2.8) strongly suggests diabetic nephropathy as the primary aetiology. Hyperkalaemia (K+ 5.6 mEq/L) is a significant concern — Metformin and Glipizide must be discontinued immediately given the impaired renal function. Normochromic anaemia (Hb 9.4 g/dL) consistent with CKD-related anaemia of chronic disease. Elevated PTH and low calcium suggest early renal osteodystrophy. Recommend: nephrology-supervised dietary protein restriction, phosphate binders, erythropoiesis-stimulating agent, and CKD-appropriate antihypertensives. Prepare patient for haemodialysis counselling.

PRELIMINARY DIAGNOSIS

Chronic Kidney Disease Stage 3b (eGFR 31). Aetiology: Diabetic Nephropathy. Associated: Anaemia of CKD, Renal Osteodystrophy, Hyperkalaemia.